جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : وداد حسن حسين شحاته
المحافظة : القاهره **الحي / المدينة :** الاميريه **العنوان :** 0
رقم البطاقة : 24912111301107 **رقم التليفون :** 01117702659 **تاريخ تسجيل الإستمارة :** 11-10-2026
العمر(تاريخ الميلاد) : 11-12-1949 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: وداد حسن حسين شحاته
National ID: 24912111301107

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





DHS

٤٥٠٢٦/٤١٢٦

هو موثق ثيمارا

GUSTAVE ROUSSY INTERNATIONAL EGYPT

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

MDT FORM

أنت المصرية هي صحة مصر

MDT FORM 17 / 9 /2025 G

Name	[illegible]	MRN	2205/2025
		ID	24912111301107
Age	45 y.o.	Phone	01177702659
Menopausal status	Post Menop-	Marital status	Married
NRS SCORE		Family history	-ve F.H
BMI		REASON OF MDT	New Case
CO morbidity	D.M		

Clinical Data

BX: IDC GII

ER:	PR:	HER 2:	Ki67:
8/8	8/8	+1	15%

R L



Sonomammography:
Rt breast multicenteric locally advanced.
Pathological ipsilat. axillary L.N Level I

Mets Workup:
CT c out Contrast → Pulm. nodules (mets)
Med. L.N 1-5 x 1-2cm

PA U/S → free.

TNM
4b1 1

Recommendations:
(Female) 45 y.o
Refer to CTs → Genetic Counseling for starting Systemic (AI).
(Zometa + chemotherapy) → AI + Index with very close f.U clinically & c CTs /3m monthly
* in Case of Progression Shift to CDK 4/6 + AI if Mets
chemo "if clinically Fixed or visceral crisis"

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: وداد حسن حسين شحاته	
MR. : 6392	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		رقم طبي:	

نشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient has metastatic breast cancer (MBC) to the lung and has been receiving an aromatase inhibitor (AI) for three months. The plan is to reevaluate for disease progression and, if progression is confirmed, to consider a shift to CDK4/6 inhibitor plus AI or chemotherapy, as discussed in the multidisciplinary team (MDT) meeting on 17-9-2025. A supportive treatment course of one month was recommended, and a CT of the chest, abdomen, and pelvis (CT CAP) with comparison was ordered.

The patient continued on AI for an additional month, but follow-up imaging was delayed due to social issues. The patient reports increasing size of the right breast mass, suggesting clinical progression. The AI was continued for one month, and follow-up CT scans were planned.

A CT CAP performed on 1-6-2026 was completed but lacked prior comparison; therefore, a repeat study with comparison was requested while the AI therapy was continued.

Imaging review showed a stationary primary breast lesion and axillary nodes, but progression of pulmonary nodules to 25 × 24 mm (previously 15 × 12 mm) and mild regression of mediastinal nodes to 10 × 12 mm (previously 19 × 12 mm). New bony metastases were identified, prompting the MDT to consider adding an anti-CDK4/6 agent.

The patient is noted to have de novo MBC to the lung, has been on AI (Femara) for six months, and a CT CAP performed on 1-6-2026 demonstrated pulmonary findings and newly developed bony lesions. Supportive treatment was continued.

At the MDT meeting on 19-7-2026, the treatment plan was updated to include Faslodex, Ibrance, and Zometa administered every six months. The patient now reports unilateral ocular pain with protrusion, inability to close the eye, and ipsilateral facial numbness, and an MRI of the brain has been ordered for further evaluation, along with a dental assessment.

تحريرا في ,

التاريخ :	19-09-2026	الوقت :	
الطبيب المعالج :	Reham Ahmed Fathy Ibrahim		

يعتمد ,

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name: Wedad Hassan Hussein Shehata	Date : 28/08/2025
Exam : ultrasound ABDOMEN& PELVIS	Age : 75 Year(s)
Medical ID:	Pat. ID: 110597

ABDOMEN AND PELVIC ULTRASONOGRAPHIC SCANNING REVEALED:

Liver is average sized displaying bright homogenous echopattern of its parenchyma. No focal lesions or intrahepatic biliary dilatation are seen.

Gall bladder multiple stones upon examination

Spleen is average sized and showing no focal lesions.

Both kidneys are of normal site, average sized, and showing homogenous echopattern. No back pressure changes. No stones or masses are seen.

Uterus is of average size. Anteverted having regular well defined outlines. The myometrium shows normal homogenous echopattern with no masses.

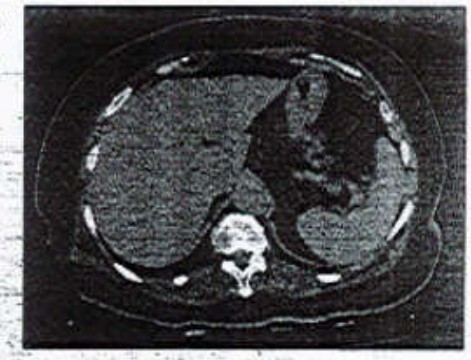
No ovarian or adenexal solid or cystic lesions.

The urinary bladder shows normal wall thickness and clear contents. No stones, masses, or diverticulae.

No ascites.

CONCLUSION:-

- ***Fatty liver.***
- ***Calcular GB***



- The liver is enlarged (19.5 cm) showing homogenous CT attenuation with no sizable focal lesions regarding this non contrast study.
- No intra-hepatic or extra-hepatic biliary radicle dilatation.
- Normal size (10 cm) and parenchymal texture of the spleen with no sizable focal lesions seen.
- Normal appearance of the pancreas and both supra renal glands.
- Normal size of both kidneys with no stones or back-pressure changes.
- No sizable abdominal or pelvic lymph nodes.
- No ascites.
- Average filling of the urinary bladder with no definite stones or masses.
- Spondylo-degenerative changes are noted.

Opinion:

- ***Hepatomegaly for clinical and lab correlation.***

Marina Essam, MSc
Dr.Noura Nabil,MD

Signature Noura Nabil
03/09/2025 3:00 AM

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY
INTERNATIONAL
EGYPT

Patient Name: Wedad Hassan Hussein Shehata	Date : 28/08/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 75 Year(s)
Medical ID: 2205/2025	Pat. ID: 110597

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS

- ***Known case of right breast cancer, no available previous films for comparison.***

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST

Suboptimal study due to inadequate breast field and respiratory motion artifact



Findings:

- The right breast shows retroareolar speculated soft tissue mass lesion measuring about +/- 3x3.5 cm associated with overlying skin thickening reaching 9 mm.
- Few suspicious looking right axillary lymph nodes are noted, the largest seen measuring 14x12.5 mm.
- Both lungs show scattered variable sized pulmonary nodules, the largest seen at the right lower lobe measuring about 15x12 mm.
- Few enlarged mediastinal lymph nodes are noted regarding this non contrast study, the largest seen at the pretracheal/retrocaval measuring about 19x12 mm.
- Patent tracheo- bronchial tree.
- Mild cardiomegaly is noted.
- No evidence of pleural or pericardial effusion.

OPINION:

- ***Right breast retroareolar speculated soft tissue mass lesion associated with overlying skin thickening.***
- ***Few suspicious looking right axillary lymph nodes.***
- ***Bilateral scattered variable sized pulmonary nodules, likely of metastatic nature.***
- ***Few enlarged mediastinal lymph nodes.***

CT SCAN OF THE ABDOMEN AND PELVIS: -

Findings:

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY

Patient Name	: الأستاذه / وداد حسن حسين شحاتة	Request No.	: 13007822
Patient No.	: 24912111301107	Request Date	: 26/07/2025
Path No.	: 2355/25	Verification Date	: 29/07/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 75 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Diagnosis
Right breast mass, Tru cut biopsy:
Invasive duct carcinoma, Grade 2
IPT:
* ER: Positive (score 8/8).
* PR: Positive (score 8/8).
* HER2: (score 1+).
* Ki67: 15 %

Signature
Dr. Walaa Ibrahim Ghoneim
Dr.Heba El-Shafeey

Verified by :
Dr.Heba Hasan

غرفة رقم (4) – الدور الاول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

USTAVE / OUSSY

Patient Name	: الأستاذه / وداد حسن حسين شحاتة	Request No.	: 13007822
Patient No.	: 24912111301107	Request Date	: 26/07/2025
Path No.	: 2355/25	Verification Date	: 29/07/2025
Sex	: Female	Financial Category	: حوستاف روسى - نفقة الدولة
Age	: 75 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data
Right breast mass, 6 o'clock lesion

Nature of specimen
Tru cut biopsy, collected on 23/7/2025.

Gross examination
Fragmented grayish white tissue cores ranging in size from 0.7 cm up to 1.5 cm, totally submitted.
IHC technique: Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination
* Histological type: Invasive duct carcinoma.
* Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2) Grade 2
* DCIS: Absent
* Angioinvasion:Positive
*Perineural invasion: Absent
*Microcalcification : Absent
- IPT (Positive control showed appropriate reactivity) :
* ER: Strong nuclear reaction in 98 % tumor cells (Allred score 8/8)
* PR: Strong nuclear reaction in 98 %tumor cells (Allred score 8/8)
* HER2: Faint incomplete membrane reaction in <10 % of tumor cells (score 1+)
* Ki 67 15 %

Verified by :
Dr Heba Hasan

غرفة رقم (4) – الدور الاول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY

Patient Name: Wedad Hassan Hussein Shehata	Date : 17/07/2025
Exam : MAMMOGRAPHY BOTH	Age : 75 Year(s)
Medical ID: 24912111301107	Pat. ID: 110597
8068	

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: 45 years old female with positive family history (maternal side aunt), complaining of left breast lump of a month duration.

Findings:

Right breast.

- Scattered fibroglandular densities. (ACR b)
- Multiple irregular speculated high density mass lesions with surrounding distortion.
- Thickened skin and coarsened trabeculae.
- Ultrasound demonstrated Multiple irregular spiculated heterogenous hypoechoic solid mass lesions with surrounding echogenic halo, posterior acoustic shadowing and mild internal vascularity on color Doppler study seen at:
  - Index Lesion: 6 o'clock, 4 cm from nipple, invading the skin measuring 30 x 27 mm.
  - 1 o'clock, 2 cm from nipple, 9 mm deep to the skin, measuring 18 x 16 mm.
  - 12 o clock, 2.7 cm from nipple, 12 mm deep to the skin, measuring 16 x 12 mm.
  - 7 o'clock, 2.2 cm from nipple, 2 mm deep to the skin, measuring 17 x 11 mm
- Thickened skin reaching 5 mm and coarsened trabeculae.
- Multiple level 1 pathological axillary LNs (more than three), largest measured 22 x 11 mm with 9 mm cortical thickness. No infra or supraclavicular lymph nodes.

Left breast.

- Scattered fibroglanduiar densities. (ACR b)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications
- Ultrasound demonstrated No solid or cystic lesions
- Retro-areolar dilated ducts reaching 4 mm with clear content/
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

Right breast (BIRADS 5): Multicentric locally advanced pathological process with ipsilateral pathological axillary lymph nodes (level 1).

Left breast (BIRADS 2): Retro-areolar duct ectasia with clear content .

Recommendation:-

For biopsy and histopathological correlation.

Best Regards,

DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations	Suspected Cancer Risk
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Patient's Name:	**Widad Hassan Hussein Shehata**
Date:	**2026-09-01**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

MRI EXAMINATION OF THE BRAIN:

MR Technique:

- Axial T1, T2 and FLAIR WI's
- Sagittal T1WI
- Coronal T2 WI
- Axial DWI and ADC.
- No contrast.

MR findings:

- Dilatation of CSF-containing structures.
- Increased peri-ventricular deep white matter signals,denoting ischemia.
- Tiny foci of altered signal are seen in the left lentiform nucleu and in both parietal lobes.No mass effects noted ,showing evidences of facilitated diffusion on DWI & ADC map respectively.
- No other cerebral, cerebellar or brain stem areas of altered MR signal or focal lesions.
- No midline shift or deformity.
- No intra or extra axial collections or masses.
- No cerebello -pontine angle masses.
- No gross posterior fossa abnormalities.
- Intact cervico- medullary junction.
- Cuts taken in the orbits revealed mildly thickened the preciliary tissue of the right orbit.
- A tiny lesion of bony signals is seen projecting in from the midfrontal cranial vault bone and measures about 6x6mm in dimensions.
- Normal the rest of the cranial vault bones and the subgaleal tissue. No fracture lines or subgaleal collection seen.
- **Cuts taken in the para-nasal sinuses revealed ethmoidal,bilateral sphenoidal ,right frontal and right maxillary sinusitis and polyps in the right maxillary antrum.**
- **Cuts taken in the rest of the orbits/sellar and para-sellar regions are unremarkable.**

IMPRESSIONS:

- **Brain involutional changes.**
- **Small artery disease.**

Patient's Name:	**Widad Hassan Hussein Shehata**
Date:	**2026-09-01**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

- **Lacunar infarcts in the left lentiform nucleus and in both parietal lobes.**
- **Mildly thickened the preciliary tissue of the right orbit.**
- **A tiny lesion of bony signals, projecting in from the midfrontal cranial vault bone,likely to be an osteoma.**
- **Otherwise,normal MRI of the brain without contrast.**

Prof/Hanan El-Ahwal.

Dr.Hanan Alahwel

Dr.Hanan Alahwel

Patient's Name:	**Widad Hassan Hussein Shehata**
Date:	**2026-09-01**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

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- **Cuts taken in the rest of the orbits/sellar and para-sellar regions are unremarkable.**

IMPRESSIONS:

- **Brain involutional changes.**
- **Small artery disease.**

جوستافروسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

09/24/2026

MDT FORM

Name:	Wedad Hassan Hussein Shehata	**MRN:**	6392
Age:	77 Y	**ID:**	
Menopausal Sta	post	**Phone:**	01117702659
NRS SCORE:	ECOG PS 2-3	**Marital Status:**	
BMI:		**Family History:**	-ve
CO Morbidity:	DM	**Reason Of MDT:**	

Clinical Data:

BX: IDC GII

ER: 8/8 **PR:** 8/8 **HER2:** +1 **KI67:** 15

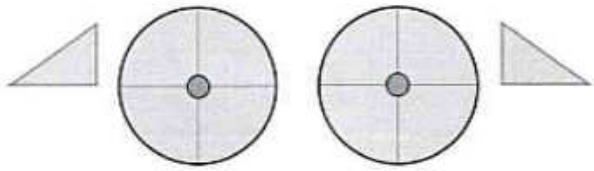
Sonomammography:

Mets Workup:

History:
denovo MBC to lung on AI 6 menthes go
CT CAP 1-6-2026 e comparison >> DP regarding pulmonary , and newly developed bony lesions

Recommendations:
according to dr Nada >> for AI + CdK 4/6 inhibitors + zometa q 6 mn

R L



T N M

MDT Outcome & Plan
for faslodex ibrance
zometa every 6 ms

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. هنان عزت					

Medical No. : 8394

Patient's Name: Widad Hassan Hussein Shehata

Date: 2026-10-05

Referred by: Prof.Dr. **no doctor**

Hospital: Goustave Roussey International Egypt

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: Pathologically proven right breast cancer, on hormonal treatment, follow up study

Findings:

Right breast.

- Scattered fibroglandular densities. (ACR b)
- Global asymmetry of almost the whole breast wit widespread distortion, diffuse breast edema pattern in the form of thickened skin and coarsened trabeculae. The over all area of pathological process measured about 9 x 6 x 4 cm.
- **Ultrasound** demonstrated pathologically proven malignant process involving mostly the whole breast in the form of non-mass altered parenchyma and multiple masses formation, the largest is LOQ invading the skin and measuring about 4 x 3.5 cm.
- Thickened skin reaching 8 mm with coarsened trabecular and minimal subdermal edema.
- Multiple (more than three) pathological axillary LNs are noted at level 1, 2 & 3 and supra-clavicular, some of axillary level 1 nodes showed extra-capsular rupture.

Left breast.

- Scattered fibroglandular densities. (ACR b)
- No masses, speculations, asymmetry or suspicious micro-calcifications.
- Benign calcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Multiple (more than three) pathological axillary LNs are noted at level 1, 2 & 3 and supra-clavicular.

Conclusion:-

Right breast (BIRADS 6): progressive pathologically proven malignant process.
Left breast (BIRADS 4a): Pathological level 1, 2, 3 axillary & supraclavicular lymph nodes.

Recommendation:-

For left axillary US-guided tru-cut biopsy and histopathological correlation.

Best Regards,

DR. Amal Hegazi. M.Sc.

Medical No. : 8394
Patient's Name: Widad Hassan Hussein Shehata
Date: 2026-10-01
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITHOUT CONTRAST

TECHNIQUE: Multi slice axial CT images of the chest, abdomen and pelvis were obtained followed by multiplanar reconstruction of the source images.

FINDINGS:

****Given history of right breast cancer:**

****AS COMPARED TO PREVIOUS STUDY DATED 1-6-2026, THE CURRENT STUDY REVELED**

NCCT CHEST:

- Progressive course in size of the multiple bilateral diffusely scattered pulmonary nodules and masses are seen, the largest one is seen at the right upper lung lobe measures about 25 x 24 mm.
- Still seen multiple enlarged mediastinal lymph nodes , the largest one measures 12 x 10 mm.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

NCCT ABDOMEN AND PELVIS:

- The liver is average in size showing homogenous CT density with no enhancing focal lesions or biliary radicles dilatation.
- The gall bladder shows average wall thickness. Its cavity shows no dense stones.
- Normal CT appearance of the spleen, pancreas and both kidneys.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- No abdominal or pelvic pathologically enlarged lymph nodes.
- No free collection.

THE SCANNED SKELETON:

- The scanned skeleton shows multiple sclerotic lesions are seen at the axial and appendicular bony skeleton more sizable at the right iliac bone.
- Fracture right iliac wing and bone with mild lateral displacement

OPINION:

Progressive course of the pulmonary metastases
Stationary rest of study
Newly formed fracture of the right iliac bone

Dr.Asser Ahmed

GUSTAVE ROUSSY

Patient Name: Wedad Hassan Hussein Shehata	Date : 17/07/2025
Exam : MAMMOGRAPHY BOTH	Age : 75 Year(s)
Medical ID: 24912111301107 8068	Pat. ID: 110597

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

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- Thickened skin and coarsened trabeculae.
- Ultrasound demonstrated Multiple irregular spiculated heterogenous hypoechoic solid mass lesions with surrounding echogenic halo, posterior acoustic shadowing and mild internal vascularity on color Doppler study seen at:
  - Index Lesion: 6 o'clock, 4 cm from nipple, invading the skin measuring 30 x 27 mm.
  - 1 o'clock, 2 cm from nipple, 9 mm deep to the skin, measuring 18 x 16 mm.
  - 12 o clock, 2.7 cm from nipple, 12 mm deep to the skin, measuring 16 x 12 mm.
  - 7 o'clock, 2.2 cm from nipple, 2 mm deep to the skin, measuring 17 x 11 mm
- Thickened skin reaching 5 mm and coarsened trabeculae.
- Multiple level 1 pathological axillary LNs (more than three), largest measured 22 x 11 mm with 9 mm cortical thickness. No infra or supraclavicular lymph nodes.

Left breast.

- Scattered fibroglandular densities. (ACR b)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- Ultrasound demonstrated No solid or cystic lesions
- Retro-areolar dilated ducts reaching 4 mm with clear content/
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

Right breast (BIRADS 5): Multicentric locally advanced pathological process with ipsilateral pathological axillary lymph nodes (level 1).

Left breast (BIRADS 2): Retro-areolar duct ectasia with clear content .

Recommendation:-

For biopsy and histopathological correlation.

Best Regards,

DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations	Suspected Cancer Risk
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	

NILE SCAN & LABS

مركز النيل للأشعة و التحاليل

Name : Widad Hassan Hussein Shehata	**Date : 05-10-2026 01:42**
Branch : Dokki	**Medical No. : 477135**
Ref.Physician Prof : Goustav Roussey Egypt	

Tc-99m MDP WHOLE-BODY BONE SCINTIGRAPHY REPORT

Clinical Indication: Routine follow-up for skeletal metastatic disease in a 77-year-old female with a history of metastatic left breast carcinoma diagnosed in 2025, on hormonal therapy, presenting with generalized bony pain.

Procedure: Whole-body anterior and posterior planar bone scintigrams were acquired following intravenous Tc-99m MDP administration.

FINDINGS

- Asymmetrical marked focal tracer concentration is seen at the right maxilla and facial bones, with scattered focal uptake throughout the calvarium. Extensive non-uniform focal lesions involve multiple cervical, thoracic, and lumbar vertebral levels. Bilateral multiple focal rib lesions, sternal tracer retention, and prominent asymmetrical pelvic uptake are noted, most accentuated at the right iliac bone and right hip region with sacral involvement.
- The appendicular skeleton shows symmetrical bilateral shoulder girdle uptake, with multiple focal lesions and heterogeneous distribution in the bilateral femora and tibiae, coexisting with age-related osteopenic features and peripheral joint degenerative uptake at both knees.
- Clear soft tissue clearance with prompt bilateral renal activity and expected bladder excretion.

IMPRESSION

- Widespread multifocal osteoblastic skeletal metastases involving the axial and appendicular skeleton, as described, superimposed on degenerative and osteopenic changes.
- Correlation with PET/CT or MRI is advised for complete staging and risk stratification.

Much obliged
Dr. Alaa Nagy, MD
Nuclear medicine

NILE DIAGNOSTIC CENTER

GUSTAVE ROUSSY
INTERNATIONAL
EGYPTE

Patient Name: Wedad Hassan Hussein Shehata	Date : 28/08/2025
Exam : ultrasound ABDOMEN& PELVIS	Age : 75 Year(s)
Medical ID:	Pat. ID: 110597

ABDOMEN AND PELVIC ULTRASONOGRAPHIC SCANNING REVEALED:

Liver is average sized displaying bright homogenous echopattern of its parenchyma. No focal lesions or intrahepatic biliary dilatation are seen.

Gall bladder multiple stones upon examination

Spleen is average sized and showing no focal lesions.

Both kidneys are of normal site, average sized, and showing homogenous echopattern. No back pressure changes. No stones or masses are seen.

Uterus is of average size. Anteverted having regular well defined outlines. The myometrium shows normal homogenous echopattern with no masses.

No ovarian or adenexal solid or cystic lesions.

The urinary bladder shows normal wall thickness and clear contents. No stones, masses, or diverticulae.

No ascites.

CONCLUSION:-

- ***Fatty liver.***
- ***Calcular GB***